# LABORATORY REPORT

Redcliffe labs

| | | | |
|---|---|---|---|
| Patient NAME | **: Mr Ravi Shankar Gunisetty** | | |
| DOB/Age/Gender | : 44 Y 2 M 3 D/Male | Report STATUS | : Final Report |
| Patient ID / UHID | : 13360594/RCL12927449 | Barcode NO | : 27442486 |
| Referred BY | : Self | Sample Type | : Whole blood EDTA |
| Sample Collected | : Aug 04, 2025, 07:22 AM | Report Date | : Aug 04, 2025, 01:42 PM. |

## CM-HC04

### Complete Blood Count (CBC)

| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|
| **RBC Parameters** | | | |
| Hemoglobin<br>*colorimetric* | 15.3 | g/dL | 13.0 - 17.0 |
| RBC Count<br>*Electrical impedance* | 5.4 | 10^6/µl | 4.5 - 5.5 |
| PCV<br>*Calculated* | 47.4 | % | 40 - 50 |
| MCV<br>*Calculated* | 87.7 | fl | 83 - 101 |
| MCH<br>*Calculated* | 28.4 | pg | 27 - 32 |
| MCHC<br>*Calculated* | 32.4 | g/dL | 31.5 - 34.5 |
| RDW (CV) *<br>*Calculated* | 12 | % | 11.6 - 14.0 |
| RDW-SD *<br>*Calculated* | 35.3 | fl | 35.1 - 43.9 |
| **WBC Parameters** | | | |
| TLC<br>*Electrical impedance and microscopy* | 7.14 | 10^3/µl | 4 - 10 |
| **Differential Leucocyte Count** | | | |
| Neutrophils | 43.8 | % | 40-80 |
| Lymphocytes | **45.5** | % | 20-40 |
| Monocytes | 4.9 | % | 2-10 |
| Eosinophils | 5.5 | % | 1-6 |
| Basophils | 0.3 | % | <2 |
| **Absolute Leukocyte Counts**<br>***Calculated*** | | | |
| Neutrophils. | 3.13 | 10^3/µl | 2 - 7 |
| Lymphocytes. | **3.25** | 10^3/µl | 1 - 3 |
| Monocytes. | 0.35 | 10^3/µl | 0.2 - 1.0 |
| Eosinophils. | 0.39 | 10^3/µl | 0.02 - 0.5 |
| Basophils. | 0.02 | 10^3/µl | 0.02 - 0.5 |
| **Platelet Parameters** | | | |
| Platelet Count<br>*Electrical impedance and microscopy* | 223 | 10^3/µl | 150 - 410 |
| Mean Platelet Volume (MPV) *<br>*Calculated* | **8.5** | fL | 9.3 - 12.1 |
| PCT * | 0.2 | % | 0.17 - 0.32 |

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| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|
| *Calculated* | | | |
| PDW *<br>*Calculated* | 10.8 | fL | 8.3 - 25.0 |
| P-LCR *<br>*Calculated* | 28.3 | % | 18 - 50 |
| P-LCC *<br>*Calculated* | 63 | 10^9/L | 44 - 140 |
| Mentzer Index *<br>*Calculated* | 16.24 | % | > 13 |

**Interpretation:**
CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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| Referred BY | : Self | Sample Type | : Whole blood EDTA |
| Sample Collected | : Aug 04, 2025, 07:22 AM | Report Date | : Aug 04, 2025, 01:59 PM. |

| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|

**Erythrocyte Sedimentation Rate (ESR)**

| | | | |
|---|---|---|---|
| ESR - Erythrocyte Sedimentation Rate<br>*MODIFIED WESTERGREN* | 4 | mm/hr | 0 - 10 |

**Interpretation:**
ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis ,systemic vasculitis, anemia, multiple myeloma , etc. Low levels are typically seen in congestive heart failure, polycythemia ,sickle cell anemia, hypo fibrinogenemia , etc.

**Reference**- Dacie and lewis practical hematology

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| Referred BY | : Self | Sample Type | : Whole blood EDTA |
| Sample Collected | : Aug 04, 2025, 07:22 AM | Report Date | : Aug 04, 2025, 02:45 PM. |

| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|

**HbA1C (Glycosylated Haemoglobin)**

| | | | |
|---|---|---|---|
| Glycosylated Hemoglobin (HbA1c)<br>*HPLC* | 5.3 | % | <5.7 |
| Estimated Average Glucose * | 105.41 | mg/dL | Refer Table Below |

**Interpretation:**
**Interpretation For HbA1c% As per American Diabetes Association (ADA)**

| Reference Group | HbA1c in % |
|---|---|
| Non diabetic adults >=18 years | <5.7 |
| At risk (Prediabetes) | 5.7 - 6.4 |
| Diagnosing Diabetes | >= 6.5 |
| Therapeutic goals for glycemic control | Age > 19 years<br>Goal of therapy: < 7.0<br>Age < 19 years<br>Goal of therapy: <7.5 |

**Note:**
1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

**Comments :**
HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

| HbA1c(%) | Mean Plasma Glucose (mg/dL) | HbA1c(%) | Mean Plasma Glucose (mg/dL) |
|---|---|---|---|
| 6 | 126 | 12 | 298 |
| 8 | 183 | 14 | 355 |
| 10 | 240 | 16 | 413 |

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| Patient ID / UHID | : 13360594/RCL12927449 | Barcode NO | : 27763146 |
| Referred BY | : Self | Sample Type | : FLUORIDE F |
| Sample Collected | : Aug 04, 2025, 07:22 AM | Report Date | : Aug 04, 2025, 02:34 PM. |

NABL M(EL)T LABS
NABL-M(EL)T-00648

| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|

## Glucose Fasting

| | | | |
|---|---|---|---|
| Glucose Fasting<br>*Hexokinase* | 84.1 | mg/dL | 70 - 100 |

**Interpretation:**

| Status | Fasting plasma glucose in mg/dL |
|---|---|
| Normal | 70 - 100 |
| Impaired fasting glucose | 101 - 125 |
| Diabetes | ≥126 |

**Reference :** American Diabetes Association

**Comment :**
Blood glucose determinations in commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy insulinoma, or various liver diseases.

**Note**
**1**.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL or a random / 2 hour plasma glucose value of > or = 200 mg/dL with symptoms of diabetes mellitus.
**2**.Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.

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| Referred BY | : Self | Sample Type | : Serum |
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| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|

## Creatinine

| | | | |
|---|---|---|---|
| Creatinine<br>*Jaffe Kinetic* | 1.1 | mg/dl | 0.70 - 1.20 |
| eGFR (CKD-EPI) | 84.78 | ml/min/1.73 sq m | Normal Or High: >= 90<br>Mild Or Decrease: 60-89<br>Mild To Moderate Decrease: 45-59<br>Mild To Severe Decrease: 30-44<br>Severe Decrease: 15-29<br>Kidney Failure: < 15 |

**Interpretation:**
Creatinine estimation is done to assess kidney function. It is not dependent on dietary factors. Normal values are obtained in kidney diseases, except in advanced renal failure and therefore its estimation is more valuable if coupled with clearance.

**"eGFR test is applicable for patients aged 18 years or more."**

## Uric Acid

| | | | |
|---|---|---|---|
| Uric Acid<br>*Uricase* | **7.5** | mg/dL | 3.4 - 7.0 |

**Interpretation:**
Serum uric acid levels are very labile and show day to day and seasonal variation in some people. Levels are also increased by emotional stress, total fasting and increased body weight. Serum uric acid levels are used to diagnose and monitor treatment of gout, monitor chemotherapeutic treatment of neoplasms to avoid renal urate deposition with possible renal failure.

## SGOT / AST

| | | | |
|---|---|---|---|
| SGOT/AST<br>*IFCC with P5P* | 33.9 | U/L | 0 - 50 |

**Interpretation:**
Serum AST is used for differential diagnosis of diseases of hepatobiliary system and pancreas. Increased values are seen in liver diseases like acute viral hepatitis, cirrhosis, biliary obstruction, primary or metastatic cancer, granuloma, hepatic ischaemia.

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| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|

**SGPT / ALT**

| | | | |
|---|---|---|---|
| SGPT/ALT<br>*IFCC with P5P* | 44.1 | U/L | 0 - 50 |

**Interpretation:**
Serum ALT is used for differential diagnosis of diseases of hepatobiliary system and pancreas. Increased in alcohalic hepatitis, cirrhosis, hepatocellular carcinoma, chronic hepatitis. Decreased in genito-urinary tract infection, malignancy, pyridoxal phosphate deficiency states (malnutrition, pregnancy, alcoholic liver disease).

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## Lipid Profile

| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|
| Total Cholesterol<br>*Enzymatic - Cholesterol Oxidase* | **247** | mg/dL | <200 |
| Triglycerides<br>*Colorimetric - Lip/Glycerol Kinase* | **203** | mg/dL | <150 |
| HDL Cholesterol<br>*Accelerator Selective Detergent* | 50.5 | mg/dL | >40 |
| Non HDL Cholesterol *<br>*Calculated* | **196.5** | mg/dL | <130 |
| LDL Cholesterol *<br>*Calculated* | **155.9** | mg/dL | <100 |
| V.L.D.L Cholesterol *<br>*Calculated* | **40.6** | mg/dL | < 30 |
| Chol/HDL Ratio *<br>*Calculated* | 4.89 | Ratio | 3.5 - 5.0 |
| HDL/ LDL Ratio *<br>*Calculated* | **0.32** | Ratio | 0.5 - 3.0 |
| LDL/HDL Ratio *<br>*Calculated* | 3.09 | Ratio | - |

**Interpretation:**
Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays.

| National Lipid Association Recommendations (NLA-2014) | Total Cholesterol (mg/dL) | Triglyceride (mg/dL) | LDL Cholesterol (mg/dL) | Non HDL Cholesterol (mg/dL) |
|---|---|---|---|---|
| Optimal | <200 | <150 | <100 | <130 |
| Above Optimal | | | 100-129 | 130 - 159 |
| Borderline High | 200-239 | 150-199 | 130-159 | 160 - 189 |
| High | >=240 | 200-499 | 160-189 | 190 - 219 |
| Very High | - | >=500 | >=190 | >=220 |

| HDL Cholesterol | |
|---|---|
| Low | High |
| <40 | >=60 |

**Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.**

| | |
|---|---|
| **Risk Category** | A. CAD with > 1 feature of high risk group |
| **Extreme risk group** | B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease |
| **Very High Risk** | 1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia |
| | 1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence |

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| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|

| | |
|---|---|
| **High Risk** | of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >/= 50 mg/dl 8. Non stenotic carotid plaque |
| **Moderate Risk** | 2 major ASCVD risk factors |
| **Low Risk** | 0-1 major ASCVD risk factors |
| **Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors** | |
| 1. Age >/=45 years in Males & >/= 55 years in Females | 3. Current Cigarette smoking or tobacco use |
| 2. Family history of premature ASCVD | 4. High blood pressure |
| 5. Low HDL | |

**Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.**

| Risk Group | Treatment Goals | | Consider Drug Therapy | |
|---|---|---|---|---|
| | LDL-C (mg/dl) | Non-HDL (mg/dl) | LDL-C (mg/dl) | Non-HDL (mg/dl) |
| Extreme Risk Group Category A | <50 (Optional goal <OR = 30) | <80 (Optional goal <OR = 60) | >OR = 50 | >OR = 80 |
| Extreme Risk Group Category B | >OR = 30 | >OR = 60 | > 30 | > 60 |
| Very High Risk | <50 | <80 | >OR = 50 | >OR = 80 |
| High Risk | <70 | <100 | >OR = 70 | >OR = 100 |
| Moderate Risk | <100 | <130 | >OR = 100 | >OR = 130 |
| Low Risk | <100 | <130 | >OR = 130* | >OR = 160 |

*** After an adequate non-pharmacological intervention for at least 3 months.**

**References : Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology,2022,20,134-155.**

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| Referred BY | : Self | Sample Type | : Serum |
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| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|
| **Vitamin D 25 Hydroxy** | | | |
| Vitamin D 25 - Hydroxy<br>*ECLIA* | **14.2** | ng/mL | Deficiency : <30 ng/mL |

**Interpretation:**
25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

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| Referred BY | : Self | Sample Type | : Serum |
| Sample Collected | : Aug 04, 2025, 07:22 AM | Report Date | : Aug 04, 2025, 03:03 PM. |

| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|
| **Thyroid Profile Total** | | | |
| Triiodothyronine (T3)<br>*ECLIA* | **65.1** | ng/dL | 80 - 200 |
| Total Thyroxine (T4)<br>*ECLIA* | 4.52 | µg/dL | 4.5 - 11.7 |
| Thyroid Stimulating Hormone (Ultrasensitive)<br>*CMIA* | **6.21** | mIU/L | 0.35 - 4.94 |

**Interpretation:**

| Pregnancy | Refrence Range TSH |
|---|---|
| 1st Trimester | 0 .1 - 2.5 |
| 2nd Trimester | 0.2 - 3.0 |
| 3rd Trimester | 0.3 - 3.0 |

**Clinical Use:**
1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

**Increased TSH:** Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance
**Decreased TSH:** Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency

Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

**1. Primary Hypothyroidism:** High TSH levels.
**2. Secondary/Tertiary Hypothyroidism:** Low TSH levels.
**3. Euthyroid Sick Syndrome:** Abnormal thyroid test results due to non-thyroidal illnesses (NTI).

TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

| TSH | T4 | T3 | Interpretation |
|---|---|---|---|
| High | Normal | Normal | Mild (subclinical) hypothyroidism |
| High | Low | Low Or Nomral | Hypothyroidism |
| Low | Normal | Normal | Mild (subclinical) hyperthyroidism |
| Low | High Or Normal | High Or Normal | Hyperthyroidism |
| Low | Low Or Normal | Low Or Normal | Nonthyroidal illness; pituitary (secondary) hypothyroidism |
| Normal | High | High | Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function) |

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|---|---|---|---|

**Estimated Glomerular Filtration Rate (eGFR)**

| | | | |
|---|---|---|---|
| Creatinine<br>*Jaffe Kinetic* | 1.1 | mg/dl | 0.70 - 1.20 |
| eGFR (CKD-EPI) | 84.78 | ml/min/1.73 sq m | Normal Or High: >= 90<br>Mild Or Decrease: 60-89<br>Mild To Moderate Decrease: 45-59<br>Mild To Severe Decrease: 30-44<br>Severe Decrease: 15-29<br>Kidney Failure: < 15 |

**Interpretation:**

1. The CKD-EPI equation, expressed as a single equation, is:

$GFR = 141 * \min(Scr/\kappa,1)^{\alpha} * \max(Scr/\kappa, 1)^{-1.209} * 0.993^{Age} * 1.018$ [if female] * 1.159 [if black]

Scr is serum creatinine (mg/dL), κ is 0.7 for females and 0.9 for males, α is -0.329 for females and -0.411 for males, min indicates the minimum of Scr/κ or 1, and max indicates the maximum of Scr/κ or 1.

2. The CKD-EPI (Chronic Kidney Disease Epidemiology Collaboration) equation was developed in an effort to create a more precise formula to estimate glomerular filtration rate (GFR) from serum creatinine and other readily available clinical parameters, especially at when actual GFR is >60 mL/min per $1.73m^2$.

**Reference:**

Levey et al. Annals of Internal Medicine 2009 May 5, 150 (9): 604-12

**"eGFR test is applicable for patients aged 18 years or more."**

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**Electrolytes (Na/K/Cl)**

| | | | |
|---|---|---|---|
| Sodium<br>*Potentiometric* | 142.5 | mmol/L | 136 - 145 |
| Potassium<br>*Potentiometric* | 4 | mmol/L | 3.5 - 5.1 |
| Chloride<br>*Potentiometric* | 101.3 | mmol/L | 98 - 107 |

**Interpretation:**
An electrolyte panel, which typically includes measurements of sodium (Na), potassium (K), and chloride (Cl) levels, is a common blood test that provides information about your body's electrolyte balance.

**1. Sodium (Na):**
1. Sodium is crucial in maintaining fluid balance in your body and is essential for nerve function and muscle contraction.
2. High sodium levels (hypernatremia) or low sodium levels (hyponatremia) can indicate various health conditions, including dehydration, kidney problems, or hormonal imbalances.

**2. Potassium (K):**
1. Potassium is vital for proper muscle function, including the heart muscle and nerve function, as well as maintaining fluid and electrolyte balance.
2. Abnormal potassium levels (hyperkalemia or hypokalemia) can indicate kidney dysfunction, dehydration, certain medications, or other underlying health issues.

**3. Chloride (Cl):**
1. Chloride works closely with sodium and potassium to maintain fluid balance and proper pH levels in the body.
2. Abnormal chloride levels may occur alongside imbalances in sodium and potassium and can indicate dehydration, kidney problems, respiratory issues, or metabolic disorders.

Healthcare providers typically order an electrolyte panel to evaluate and monitor electrolyte imbalances, diagnose certain medical conditions, assess kidney function, or monitor treatment effectiveness. It's usually part of routine blood testing, especially in cases of dehydration, kidney disease, heart conditions, or electrolyte disorders.

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| DOB/Age/Gender | : 44 Y 2 M 3 D/Male | Report STATUS | : Final Report |
| Patient ID / UHID | : 13360594/RCL12927449 | Barcode NO | : 27513377 |
| Referred BY | : Self | Sample Type | : Spot Urine |
| Sample Collected | : Aug 04, 2025, 07:22 AM | Report Date | : Aug 04, 2025, 02:33 PM. |

NABL M(EL)T LABS
NABL-M(EL)T-00648

| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|

**Urine Routine and Microscopic Examination**

| | | | |
|---|---|---|---|
| **Physical Examination *** | | | |
| Volume * | 20 | ml | - |
| Colour * | Pale yellow | - | Pale yellow |
| Transparency * | Clear | - | Clear |
| Deposit * | Absent | - | Absent |
| **Chemical Examination *** | | | |
| Reaction (pH)<br>*Double Indicator* | 5.0 | - | 4.5 - 8.0 |
| Specific Gravity<br>*Ion Exchange* | 1.025 | - | 1.010 - 1.030 |
| Urine Glucose (sugar)<br>*Oxidase / Peroxidase* | Negative | - | Negative |
| Urine Protein (Albumin)<br>*Acid / Base Colour Excahnge* | Negative | - | Negative |
| Urine Ketones (Acetone)<br>*Legals Test* | Negative | - | Negative |
| Blood<br>*Peroxidase Hemoglobin* | Negative | - | Negative |
| Leucocyte esterase<br>*Enzymatic Reaction* | Negative | - | Negative |
| Bilirubin Urine<br>*Coupling Reaction* | Negative | - | Negative |
| Nitrite<br>*Griless Test* | Negative | - | Negative |
| Urobilinogen<br>*Ehrlichs Test* | Normal | - | Normal |
| **Microscopic Examination *** | | | |
| Pus Cells (WBCs) * | 2-3 | /hpf | 0 - 5 |
| Epithelial Cells * | 1-2 | /hpf | 0 - 4 |
| Red blood Cells * | Absent | /hpf | Absent |
| Crystals * | Absent | - | Absent |
| Cast * | Absent | - | Absent |
| Yeast Cells * | Absent | - | Absent |
| Amorphous deposits * | Absent | - | Absent |
| Bacteria * | Absent | - | Absent |
| Protozoa * | Absent | - | Absent |

**Interpretation:**
**URINALYSIS-** Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders.

**Protein:** Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

Dr. Lakshmi Kasula
MD (Path)
Consultant Pathologist

| | | | |
|---|---|---|---|
| Patient NAME | **: Mr Ravi Shankar Gunisetty** | | |
| DOB/Age/Gender | : 44 Y 2 M 3 D/Male | Report STATUS | : Final Report |
| Patient ID / UHID | : 13360594/RCL12927449 | Barcode NO | : 27513377 |
| Referred BY | : Self | Sample Type | : Spot Urine |
| Sample Collected | : Aug 04, 2025, 07:22 AM | Report Date | : Aug 04, 2025, 02:33 PM. |

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| Test Description | Value(s) | Unit(s) | Reference Range |
|---|---|---|---|

**Glucose:** Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications.

**Ketones:** Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise.

**Blood:** Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders.

**Leukocytes:** An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection.

**Nitrite:** Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection.

**pH:** The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine.

**Specific gravity:** Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus.

**Bilirubin:** In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine.

**Urobilinogen:** Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.

*** End Of Report ***

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

Dr. Lakshmi Kasula
MD (Path)
Consultant Pathologist

# HOW WAS YOUR EXPERIENCE?

We value your feedback, please scan with your phone camera
and rate overall experience.

How likely are you to recommend Redcliffe Labs to your family & friends

| 1 | 2 | 3 | 4 | 5 | 6 | 7 | 8 | 9 | 10 |
|---|---|---|---|---|---|---|---|---|---|

Not Likely — Very Likely

**Scan with your phone camera**